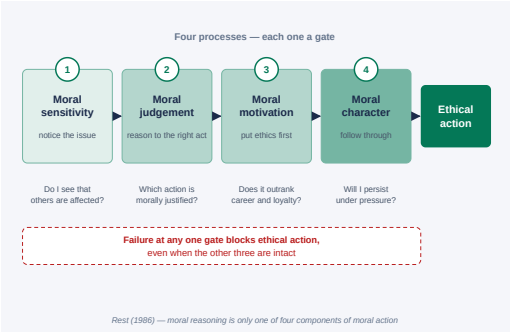
Unit 5 Frameworks

8 frameworks from Slibrary 9 to 10, each with its figure, Harvard reference, a note on the author(s) and the guiding question for the Vireon Diagnostics case.

Slibrary 9 — Ethics & Moral Courage

4 frameworks



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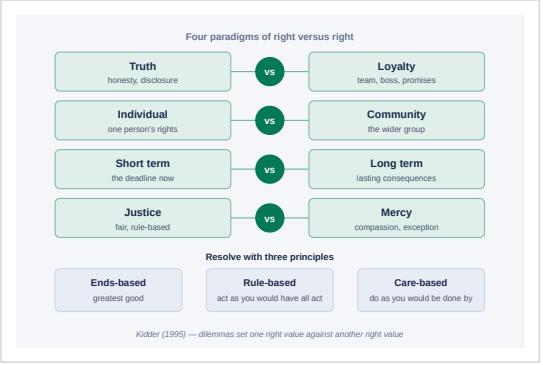
Rest's Four-Component Model

Ethical action passes four gates — or fails at one

Source (Harvard): Rest, J.R. (1986) Moral Development: Advances in Research and Theory. New York: Praeger.

In one sentence	Acting ethically takes four separate steps — noticing the ethical issue, judging what is right, caring about it more than competing goals, and having the strength to follow through; failing at any one step stops the action.
About the author(s)	James R. Rest (1941–1999) was a professor of educational psychology at the University of Minnesota. He developed the Defining Issues Test, a widely used measure of moral judgement, and was a leading figure in the neo-Kohlbergian school of moral psychology.
Guiding question (Vireon Diagnostics case)	*At Vireon Diagnostics, Ferraro, Bellandi and two uneasy team members all know about the 88% figure for patients over 80: at which of Rest's four gates is each of them most likely to stop?*

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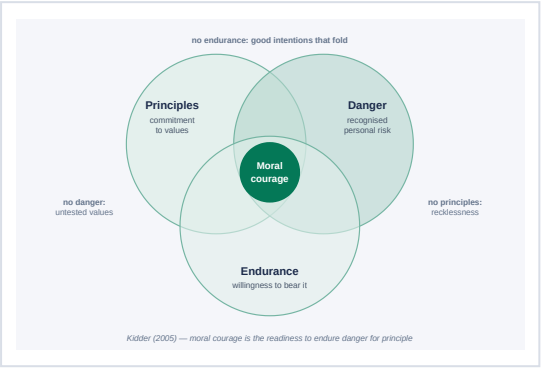
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Kidder's Right-vs-Right Paradigms

When both options are right, name the values in conflict

Source (Harvard): Kidder, R.M. (1995) How Good People Make Tough Choices: Resolving the Dilemmas of Ethical Living. New York: William Morrow.

In one sentence	Many hard decisions are not between right and wrong but between two rights; naming which pair of values collides helps leaders reason openly and choose a course they can defend.
About the author(s)	Rushworth M. Kidder (1944–2012) was an American author and former columnist for The Christian Science Monitor. In 1990 he founded the Institute for Global Ethics, through which he ran ethics programmes for companies, schools and governments worldwide.
Guiding question (Vireon Diagnostics case)	*Ferraro must choose between a 1 March go-live worth €3.8 million and seven more weeks to fix the 88% result for patients over 80: which of Kidder's paradigms are in play, and which side would each resolution principle favour?*



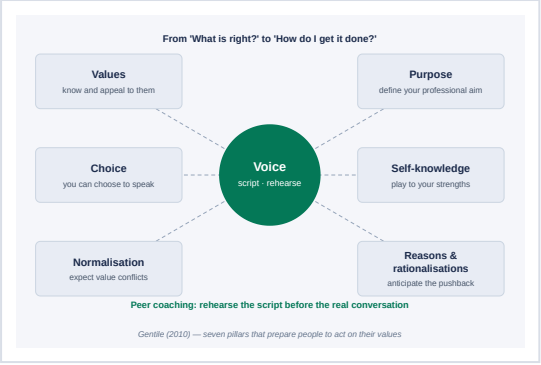
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Kidder's Moral Courage Model

Principles, danger and endurance — courage needs all three

Source (Harvard): Kidder, R.M. (2005) Moral Courage. New York: William Morrow.

In one sentence	Moral courage appears only when three things meet: a clear commitment to a principle, an honest view of the personal risk involved, and the willingness to endure that risk.
About the author(s)	Rushworth M. Kidder (1944–2012) was an American author, ethicist and founder of the Institute for Global Ethics (1990). Moral Courage (2005) followed his earlier work on ethical dilemmas and drew on stories of people across business, politics and public life.
Guiding question (Vireon Diagnostics case)	*Ferraro admires Bellandi, who promoted her, and his bonus depends on the launch date: in the next 48 hours, which of the three elements of moral courage is weakest for her, and how could she strengthen it?*



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Giving Voice to Values

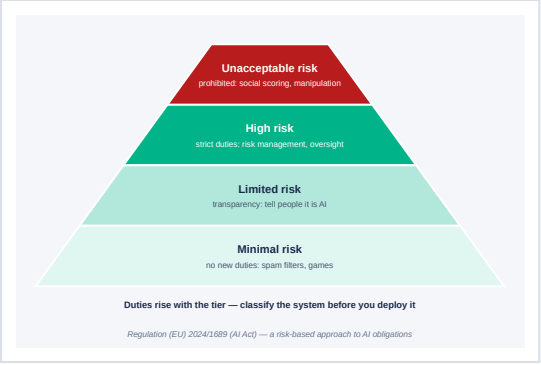
Script, rehearse and voice what you already know is right

Source (Harvard): Gentile, M.C. (2010) Giving Voice to Values: How to Speak Your Mind When You Know What's Right. New Haven, CT: Yale University Press.

In one sentence	Most people know what is right but struggle to say it; Giving Voice to Values prepares them to speak up by anticipating objections, writing a short script and rehearsing it with peers.
About the author(s)	Mary C. Gentile is a business-ethics educator who created the Giving Voice to Values curriculum. Earlier she worked at Harvard Business School on its ethics curriculum; GVV is now used in business schools and companies in many countries.
Guiding question (Vireon Diagnostics case)	*Bellandi will argue that 96% beats the manual process and that twenty jobs in Lyon are at stake: what are his strongest rationalisations, and what would Ferraro's 60-second opening script say in response?*

Slibrary 10 — Trustworthy AI & Accountability

4 frameworks



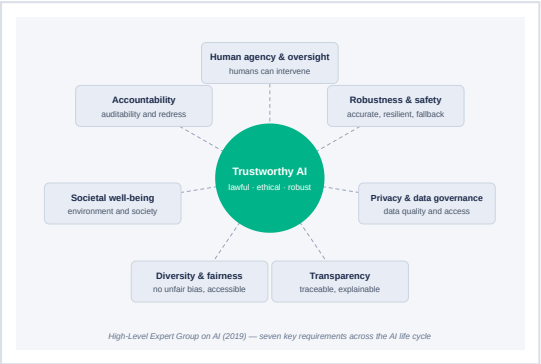
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EU AI Act Risk Pyramid

Obligations scale with the risk an AI system poses

Source (Harvard): European Parliament and Council of the European Union (2024) Regulation (EU) 2024/1689 laying down harmonised rules on artificial intelligence (Artificial Intelligence Act). Official Journal of the European Union, L, 12 July 2024.

In one sentence	The EU AI Act sorts AI systems into four risk levels — banned, high, limited and minimal — and the higher the risk, the more duties the organisation must meet before and while using the system.
About the author(s)	The European Commission proposes EU legislation; the European Parliament and the Council of the EU, representing citizens and member-state governments, adopt it. The AI Act was negotiated between 2021 and 2024 and is supervised by national authorities and the European AI Office.
Guiding question (Vireon Diagnostics case)	*TriageAssist ranks blood results by urgency and is likely to be a high-risk system: which obligations of that tier does the 88% result for patients over 80 touch, and what does that mean for a 1 March go-live?*



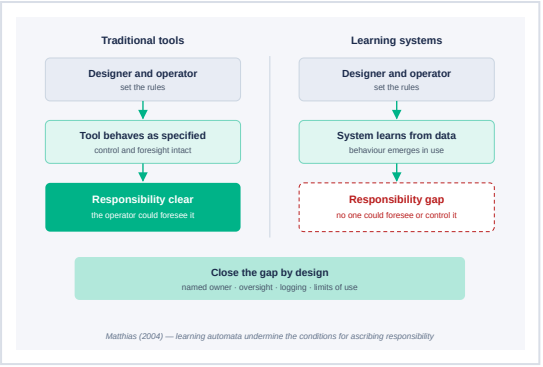
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Seven Requirements for Trustworthy AI

Seven conditions that make AI lawful, ethical and robust

Source (Harvard): High-Level Expert Group on Artificial Intelligence (2019) Ethics Guidelines for Trustworthy AI. Brussels: European Commission.

In one sentence	Trustworthy AI should meet seven conditions — human oversight, robustness, privacy, transparency, fairness, social well-being and accountability — and neglecting any one of them undermines trust in the whole system.
About the author(s)	The High-Level Expert Group on Artificial Intelligence (2018–2020) advised the European Commission on its AI strategy. Its members came from academia, industry and civil society. Besides the ethics guidelines it produced policy and investment recommendations and the ALTAI assessment list for organisations.
Guiding question (Vireon Diagnostics case)	*Physicians at the Lombardy hospitals would rely on TriageAssist's ranking: which of the seven requirements does the weaker result for patients over 80 put most at risk, and what evidence would show it is met?*



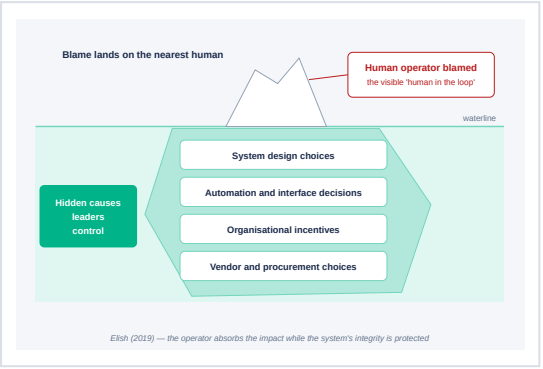
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The Responsibility Gap

When machines learn, who answers for what they do?

Source (Harvard): Matthias, A. (2004) 'The responsibility gap: ascribing responsibility for the actions of learning automata', Ethics and Information Technology, 6(3), pp. 175-183.

In one sentence	When a learning system behaves in ways its makers could not predict or control, it becomes unclear who is responsible for harm — so leaders must assign responsibility deliberately before something goes wrong.
About the author(s)	Andreas Matthias is a philosopher working on the ethics of technology and artificial intelligence. His 2004 article introduced the term 'responsibility gap', which has become a central reference in debates on AI accountability and autonomous systems.
Guiding question (Vireon Diagnostics case)	*If a critical result for an 85-year-old patient is ranked low and treatment is delayed, who at Vireon — Ferraro, Bellandi, the data scientist or the physician — could fairly be held responsible, and how could the gap be closed before 1 March?*



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Moral Crumple Zone

Blame lands on the nearest human, not on the system

Source (Harvard): Elish, M.C. (2019) 'Moral crumple zones: cautionary tales in human-robot interaction', Engaging Science, Technology, and Society, 5, pp. 40-60.

In one sentence	When an automated system fails, people tend to blame the human operator nearest to it, even when the real causes lie in design and organisational decisions the operator could not control.
About the author(s)	Madeleine Clare Elish is a cultural anthropologist who studies the social impacts of AI and automation. She led research at the Data & Society Research Institute in New York and has since worked on responsible AI in industry.
Guiding question (Vireon Diagnostics case)	*Bellandi says 'physicians still review every result': if an urgent result for a patient over 80 is missed, why might the reviewing physician become the moral crumple zone, and which of Vireon's decisions would sit below the waterline?*